

Standard Operating Procedure

Incident Reporting, Analysis and Root-Cause Investigation

«Hospital Name» — Supports PSQ.7.a-e (CORE)

SOP No.	[Hospital to define]	Version	1.0
Effective Date	_____	Review Frequency	Annual, or on change
Prepared By Quality Coordinator Name: _____ Signature: _____			
Approved By Medical Superintendent Name: _____ Signature: _____			

1. Purpose

This SOP describes how «Hospital Name» identifies, reports, reviews and acts on every patient safety incident — from no-harm events to sentinel events — and how sentinel events are handled against a strict clock.

2. Scope

Applies to every incident, near-miss, adverse event and sentinel event, anywhere in the hospital, regardless of severity or whether harm was caused.

3. Responsibility

Any Staff Member: reports the incident on the standardised form the same shift it is noticed.

Safety Committee: analyses incidents and owns the correction/analysis timeline for sentinel events.

Patient Safety Officer: coordinates the incident management system.

4. Procedure

1. Identify the event against the hospital's definitions. A near-miss is an unplanned event that did not result in injury, illness or damage, but had the potential to. A no-harm event is one where an error occurred but was caught before it reached the patient and no injury resulted. An adverse event is an injury related to medical

management — diagnosis, treatment, or the systems and equipment used to deliver care — rather than to the patient's underlying disease. A sentinel event is an unanticipated occurrence involving death or serious physical or psychological injury, not related to the patient's underlying illness, but related to the medical equipment, supplies or care provided (NABH Guidebook, Glossary of Terms).

2. Report every incident on the standardised incident report form, capturing all incidents without first deciding severity or whether harm occurred — that judgement comes later, during analysis, not at the point of reporting.

3. Keep the reporting system simple, clear and confidential, and focused on process improvement rather than blame — this is what keeps a just culture reporting honestly (see the Safety Culture policy, PSQ.6).

4. Address the immediate care and support needs of everyone involved first — this does not wait for analysis to finish.

5. Analyse the incident. The Safety Committee is responsible for this, using root-cause analysis: a structured, system-based review that identifies the root and contributory factors behind what happened, sets out risk-reduction strategies, and builds an action plan with a way to measure whether the plan actually worked. Seek input from the units, disciplines or departments concerned, and include patients and other stakeholders in the analysis where possible.

6. For a sentinel event specifically: start correction within 24 working hours of the event occurring or being reported, and complete the full analysis within 7 working days of occurrence or reporting. Track both deadlines in the Sentinel Event Timeline Compliance Log (Record 4) — missing either deadline is itself a finding that needs its own corrective action.

7. Document corrective and preventive action based on the findings. Communicate findings and recommendations to everyone who needs to act on them, so systems and processes actually change — not just the report.

8. Reflect any resulting change to policy or procedure as a formal amendment to that document, not as a side note.

9. Where analysis reveals a previously unidentified risk, add it to the hospital's risk management system — an incident that surfaces a new risk should leave the hospital safer than before the incident happened.

10. After analysis, classify the event as a near-miss, adverse event, or sentinel event, and inform the relevant stakeholders — including the patient and family where

applicable — about what happened and what is being done, alongside starting the corrective action.

5. Stop-Work Authority

Missing the sentinel-event correction deadline (24 working hours) or the analysis deadline (7 working days) is a CORE non-compliance in its own right. Any staff member who identifies that either deadline has been missed escalates to the Patient Safety Officer immediately — raising a missed deadline is never a disciplinary matter.

6. Records

Supported by Records 1-5 of the PSQ.7 Required Records pack — the incident report log, sentinel event identification list, incident analysis/RCA record, sentinel event timeline compliance log, and the CAPA record.

7. Related Documents

Policy on Incident Management and Safety Events (HCO/PSQ/POL/07) — PSQ.7 policy

Policy on Patient Safety Programme (HCO/PSQ/POL/01) — PSQ.1 policy, Section 3.3 (no-harm and sentinel event coverage)

8. Review

Reviewed annually, or immediately after any sentinel event reveals a gap in this procedure.

9. References

National Accreditation Board for Hospitals and Healthcare Providers. Guidebook to NABH Accreditation Standards for Hospitals, 6th Edition. Glossary of Terms — near-miss, no harm, adverse event, sentinel event.

American Society for Quality. (n.d.). What is Root Cause Analysis (RCA)?

<https://asq.org/quality-resources/root-cause-analysis>